

RaphCare progress (partner view)

This is the **simple** status board. Same topics as the builder checklist, without code names.

Overall completion: 81%

How to read this

- **Done** = 100% of that row
- **Partial** = 50%
- **Not started** = 0%
- **Not for mobile**, **Not for clinical v1**, and **Not for BLE** = left out of the % on purpose
- **Later** notes (phone use of the web admin, and similar) are left out of the % until we schedule the work
- Section % = average of that section's scored rows
- Overall % = average of all scored rows in sections 1-5
- **Section 6** is a later product list. Those rows stay out of overall % until we schedule them and change the status to Partial or Done

Each section shows its % in the heading. Numbers stay in sync when status changes.

Last reviewed: 2026-09-04

1. Hospital admin (outpatient) - 100%

What staff do in the **web admin panel** for day-to-day clinic work (not overnight beds).

Area	Status	What you can do
Hospital list, register, claim	Done	Find or register a hospital. Each hospital gets a short RaphCare reference staff can share.
Opening a hospital	Done	The left menu hides so the hospital tools have more room. Use All hospitals in the top bar to go back. Language and sign out are there too.
Hospital profile and facilities	Done	Edit details; add physical or virtual locations. The hospital page opens with a large header and a section menu on the side.
Staff invites and roles	Done	Invite people. Set staff, doctor, pharmacist, or lab technician. Admins can still do hospital setup.
Patients at a hospital	Done	Grant or remove access; open the patient chart
Patient chart (view)	Done	Same large header and section menu as other hospital pages. Sections: overview, clinical, coverage, devices, and care. If the patient uploaded a photo on the phone, it shows in that header. Staff view this; they do not edit it here.

Clinical team and schedules	Done	Add clinicians; manage schedules. Clinician detail uses the same large header and section menu.
Appointments	Done	Book, cancel, reschedule. The appointments page uses the same large header and section menu.
Visits and vitals	Done	Start a visit; record vitals and notes; order prescriptions and lab tests while the visit is open; complete. Visit and tele join pages match the hospital page layout.
Collection counter	Done	Search by pickup code, name, or health ID. Scan a QR from the camera. Call a code onto the waiting screen. Mark a prescription collected. Enter a lab result. Cancel or undo. Print a slip or a wall poster with a QR code. Open a waiting screen on a TV or tablet (codes only, no names). The page uses the same large header and section menu as the hospital details page. Any hospital staff can do this, including after the visit is closed. Pharmacists collect medicines. Lab technicians enter lab results. Generic staff can still do both.
Video join for staff	Done	Start a tele session from admin
Dashboard numbers	Done	See clinic metrics
Web languages (English, French, Lingala, Swahili)	Done	Change language in the admin portal
Same tools inside the patient phone app	Not for mobile	Patients use the app; staff use the web admin

Bottom line (100%): outpatient hospital admin is ready on the web, including a patient chart staff can read, visit notes doctors can add while a visit is open, a collection counter for medicines and lab tests, and a waiting screen that shows pickup codes (not names). Opening a hospital hides the left menu so staff can focus on that hospital.

Later (not in this %): staff should use this same web admin on a phone. Make the screens work on a small display first. A light home-screen shortcut can follow. A full offline installable web app is not the plan.

2. Inpatient (wards, beds, admit and discharge) - 100%

Overnight stays: set up rooms and beds, put a patient in a bed, discharge them.

Area	Status	What you can do
See bed board (how many free or full)	Done	Open Inpatient on a hospital in admin. The page uses the same large header as hospital details, with a section menu on the side. Beds on the ward map look like simple top-down beds.
Add ward, room, bed	Done	Admins can create capacity

Admit a patient to a bed	Done	Search patients and pick an available bed
Discharge (free the bed)	Done	End an active stay with optional notes
Edit or remove wards, rooms, or beds later	Done	Edit, deactivate, or delete; archives if history exists
Mark a bed "maintenance"	Done	Toggle Available or Maintenance on free beds
Move a patient to another bed	Done	Transfer on the active admissions board
Past admission history	Done	Filterable history list with load more
Same board in the phone app	Not for mobile	Unless we later build a clinician app

Bottom line (100%): inpatient lifecycle is complete in admin web (capacity, maintenance, transfer, history).

3. Patient phone app - 59%

What patients see on Android and iPhone.

Screens and features

Area	Status	Notes
Sign up or sign in (email, phone code, voice)	Done	Includes forgot password: email code, then new password
Home and navigation	Done	Monochrome icons (same teal tint style as the web). Failed taps should not close the app. Some areas can be turned off with feature flags
Appointments (list, book, detail)	Done	Book uses your clinic and a clinician name list
Care, request a call, join video	Partial	Video call works on Android ; iPhone video still needs more setup
Health records	Done	Includes pickup codes and a QR code for medicines and lab tests waiting at the hospital. Scan a wall poster at the counter to show only that hospital. When staff call your code, you get a notice and Health records say come to the counter.
Insurance	Done	
Billing and payment methods	Done	

Family members	Done	
Mental health (mood check-in)	Done	
Chat assistant	Partial	Works; real smart replies need cloud AI keys in production
Notifications list	Done	Real push alerts need Firebase and store setup
Settings and profile	Done	Includes My clinic: search by name or enter a short clinic code
Choose my clinic	Done	Patients do not need Guids; use clinic name or an RC- code from the hospital

Phones and devices (broader coverage)

Area	Status	Notes
Run on Android phones	Done	Main day-to-day test target
Host API and admin web for remote Android testers	Partial	API and admin web App Services exist; admin web Staging settings point at the hosted API; finish deploy smoke and Play invite
Play Store internal test package id	Partial	Android id set to a stable Yindula id; upload via Play Console still needed
Run on iPhone	Partial	App can target iOS; full release checks still open
Same feature checked on both Android and iPhone before "ready"	Not started	Use the release checklist when locking a release
Video visit on a real Android phone	Partial	Wiring is in; confirm on hardware
Video visit on a real iPhone	Not started	Needs iOS video kit wiring
Push alerts on Android	Partial	Needs Firebase configured on the server
Push alerts on iPhone	Partial	Needs store and push setup, then a real device test
Connect E580 or E585 style Bluetooth bands	Partial	Scan includes ET580 and ET585 labels; heart rate or oxygen only when the band speaks standard Bluetooth health profiles
Live heart rate and blood oxygen in the app	Partial	Android vendor path wired; confirm on your ET580 or ET585 samples
Auto sync and background monitoring	Not started	Manual sync exists; background sync not finished
Full band health history (like the vendor companion app)	Partial	Connect and live HR/SpO ₂ started; activity, sleep, and history still open
Activity (steps, calories, distance), sleep, stress	Not started	On the watches; not in RaphCare yet

Body temp, ECG, glucose-style screens from the band	Not started	Documented; clinical use of optical glucose stays gated
Weather and lifestyle pushes to the watch	Not for clinical v1	Watch needs an app for weather; not a patient vital
Full vendor band SDK (HBand) on Android	Partial	Download libs and app bridge in place; hardware prove-out next
Full vendor band SDK on iPhone	Not started	Android first
Y6 Pro emergency 4G watch flow	Partial	Webhook, SMS, clinic patient chart, and hospital Devices emergency board; OEM mapping and push alerts still future
Windows PC build of the app	Not for BLE	Bluetooth patient devices are for phones, not the Windows target

Wearable feature list: we keep a separate model-agnostic list of what a band or watch should support, even when the hardware SKU changes.

Bottom line (59%): the patient app screens are largely there, including choosing a clinic by name or short code. Home and Profile icons are monochrome now. Catch-up work is mostly **iPhone video**, **real push**, **wearables depth** (live vitals, full sync, background), and **testing the same flows on both phone types**.

4. Other staff and system pieces - 86%

Area	Status	Notes
Core clinical and patient APIs	Done	Power the app and admin
FHIR export	Done	For interoperability
Emergency webhook	Done	Integration hook
Staging demo sign-in accounts	Done	Staging only. Sign in with demo.admin, demo.doctor, demo.pharmacy, demo.lab, or demo.patient at raphcare.com. Password is on the API as Demo:Password (default RaphCareDemo!2026). Does not wipe other hospitals.
Patient photos and voice files	Done	Photos from the phone and voice sign-up audio are stored privately. They stay after the website restarts.
Staff mental-health assessment store	Partial	List exists; deeper storage later
Reporting and big dashboards	Partial	Grows with product needs

Bottom line (86%): core APIs, hooks, Staging demo accounts, and private file storage are in; deeper mental-health storage and reporting grow with product needs.

5. Hospital plan extras - 100%

What a site gets when they pay for Hospital, not Clinic. Beds and the collection counter were already in. This step is the rest of the stay: ward notes, a bill at discharge, occupancy numbers, a nurse job, a casualty queue, today's theatre list, outbound referrals, SafeCare alerts on the hospital home, booking a return visit when someone leaves, who is on today, and an AI draft of the discharge summary.

Area	Status	What you can do
Ward notes on the stay	Done	A nurse, doctor, or hospital admin can add a note (and optional heart rate, temperature, or oxygen) while the person is in a bed.
Bill at discharge	Done	On discharge, enter a nightly bed rate and any extra charge. Print that as an invoice. Mark paid in cash.
Discharge summary on the phone	Done	The summary the hospital writes shows in the patient's Health records as a stay.
Occupancy numbers	Done	See occupancy %, how many people came in or left today, and average length of stay.
Nurse job	Done	Invite someone as a nurse. They can write ward notes. They cannot document an outpatient visit.
Lab result on the phone	Done	A completed lab shows on that visit's health record. The patient also gets an in-app notice (and push when configured) that the result is ready. The notice does not include the result values.
Casualty and triage queue	Done	Add a walk-in with a priority colour. Call a code onto a second waiting screen. The TV shows codes only, not names.
Theatre list	Done	Put today's operations on one board. Start, complete, or cancel a case.
Emergency list on the hospital home	Done	Recent SOS and fall alerts show on the hospital overview and the admin home. Full history stays on Devices.
Follow a referral through	Done	Log an outbound referral, then mark it accepted, completed, or cancelled.
Book a return visit at discharge	Done	On discharge, optionally pick a clinician and time for the next visit before the person leaves.
Who is on today	Done	Pick a day and see who is on Morning, Afternoon, or Night. Add or remove staff for a shift.
Draft discharge summary with AI	Done	On discharge, tap Draft with AI when the hospital allows it (toggle under Organization details). Staff edit the text, then save. The draft uses stay reason and ward vitals only, not free-text ward notes. Needs cloud AI keys on the server; otherwise drafting is unavailable.

Bottom line (100%): Hospital plan stay extras for this board are in. Ward notes, cash invoice, occupancy, nurse job, lab result-ready notice, casualty, theatre, referrals, SafeCare on the hospital home, return visit at discharge, who-is-on-today, and AI discharge draft.

Later (not in this %): staff using the same web admin on a phone. Same note as section 1.

6. Next ideas (AI and clinic ops) - later (not in overall %)

Ideas for after the Hospital plan board. AI still only drafts or suggests. A person always checks before anything is saved. A hospital can turn AI drafting off. We are not building US insurance form-fighting or an AI that acts as the patient's only doctor.

Area	Status	What you could do later
Open work board	Later	One place for overdue referrals, missed return visits, medicines not yet collected, and labs still waiting
Smart suggestions on that board	Later	Soft prompts for what staff might do next. Staff confirm.
Visit or ward note draft with AI	Later	Same idea as the discharge draft: start text from vitals and stay facts. Staff edit.
Reminders for visits and pickup	Later	In-app first. Text or call reminders when the site is set up for that.
Plain-language help on labs and vitals	Later	The phone chat explains results and band readings in simple words, and tells people to ask their clinician when unsure
Lab wording help	Later	Suggest how to phrase a lab result and flag a big change from the last one. Staff edit values.
Casualty priority hint	Later	Suggest a colour priority from the complaint and vitals. Staff confirm before the queue.
Theatre and who-is-on checks	Later	Warn when cases or shifts clash. Light suggestions only.
Referral from a photo or paper	Later	Snap or upload a letter. Get a draft referral to edit and save.
Busy counter hints	Later	Suggest when the collection counter may be busy.
Discharge checklist on the phone	Later	After leaving: medicines, return visit, and warning signs in plain language
Band and stay trend alerts	Later	Alert staff if heart rate or oxygen drifts from the last ward check. No automatic orders.
Invoice line suggestions	Later	At discharge, suggest bed nights and extras from the stay. Cash invoice only.
Staff license reminders	Later	Remind when a doctor or nurse license date is near.

Imaging report draft (partner)	Later	Optional link for X-ray style draft reports. Radiologist edits. Not built in-house first.
Pull records from other systems	Later	Only if a site needs outside charts.
Deeper mental-health note help	Later	Only if staff mental-health storage grows.
US insurance prior auth and claims	Not for this product	Out of scope for RaphCare.

Bottom line (later): a backlog for the next product wave. Prefer order when you schedule: visit or ward note draft, open work board, plain-language labs help, reminders, then casualty priority hint.

Rollup

Area	%	Notes
1. Hospital admin (outpatient)	100%	Ready on web
2. Inpatient	100%	Ready on web
3. Patient phone app	59%	iPhone video, push, wearables
4. Other staff and system	86%	Demo accounts on Staging; private photos and voice files
5. Hospital plan extras	100%	Stay extras closed for this board
6. Next ideas (AI and ops)	n/a	Later list; not in overall %
Overall	81%	Sections 1-5. Excludes "not for mobile, clinical, or BLE" and section 6 later rows

What to try this week (partner)

This list is the current manual smoke for admin web and the patient phone app. Builders also keep a longer layered plan (API first, then web automation, then phone) for when scripted smoke is added.

1. **Admin web:** sign in as demo.admin@raphcare.com (password RaphCareDemo!2026 unless you changed Demo:Password). Open **RaphCare Demo Clinic**.
2. **Admin web:** the left menu should hide on a hospital. Use All hospitals in the top bar to return to the list.
3. **Admin web:** on that hospital, open Inpatient. Add a ward, room, or bed if needed, admit someone. Add a ward note. On discharge, try Draft with AI, edit the text, then finish with a nightly rate and (optional) a return visit for next week. Confirm occupancy numbers on the hospital dashboard.

4. **Admin web:** open Casualty. Add a walk-in (with or without a patient). Call the code and open the casualty waiting screen on another tab. Open Theatre and add today's case, then start and complete it. Open Referrals, log a referral, then accept or complete it. Open Roster, add someone to Morning or Afternoon, then remove them if you want.
5. **Admin web:** on the hospital overview (and the admin home when that hospital is selected), confirm recent SOS or fall alerts show if any exist. Open Devices for the full list.
6. **Admin web:** open a patient chart. Use the section menu for overview, clinical, coverage, devices, and care. If that patient uploaded a photo on the phone, it should show in the header.
7. **Admin web:** book an appointment and start a visit (outpatient path). While it is in progress, add a note, a prescription with more than one medicine if you want, or a lab order (not a result).
8. **Admin web:** open Collection. Search by pickup code, name, or health ID, or scan a pickup QR. Call a code onto the waiting screen. Open the waiting screen on another tab. Mark a prescription collected, or enter a lab result. Try cancel and undo. Print a slip and a wall poster.
9. **Patient app on Android:** sign in as `demo.patient@raphcare.com`. You should get a notice that something is ready to collect. After staff enter a lab result, you should also get a separate notice that the result is ready (open Health records for the values). Waiting items show a pickup code and a QR code. Scan the wall poster at the counter to show only that hospital. When staff tap Call, the app should say come to the counter and you should get a notice.
10. **Patient app on Android:** open Home, try Appointments and Devices (Bluetooth band if you have one).
11. **Patient app on iPhone (if available):** same smoke test; note anything that fails (especially video).